■ **BOARD CONFIDENTIAL — CEO EYES ONLY**

**TO:** Ms. Shobana Kamineni, Executive Vice Chairperson | **FROM:** Strategy Office | **DATE:** 11 April 2026 | **REF:** APEX-2026-01

# Apollo 24|7 — The Decision You Need to Make in Q2 FY27

*A two-page brief. Three decisions. One signature.*

THE BOTTOM LINE

**Apollo 24|7 crossed the economic turning point in Q3 FY26. Losses have fallen 55% in two quarters (₹65 Cr → ₹29 Cr), CAC payback is now 27 months, and LTV/CAC is 9x. The business no longer needs more conviction — it needs a binding gate, the two right KPIs, and a pre-committed exit path if it wobbles. Cap remaining burn at ₹450 Cr across FY27–FY29 and reverse-merge into HealthCo NewCo if Q2 FY27 gate fails.**

## WHAT CHANGED (AND WHY IT MATTERS NOW)

| ₹29 Cr | 27 mo | 9x | 46 M | +28% |
|---|---|---|---|---|
| Q3 CASH LOSS (LOWEST EVER) | CAC PAYBACK (WAS 240) | LTV / CAC (WAS <1X) | REGISTERED USERS | GMV YOY (Q3) |

Two quarters ago, 24|7 was a venture-stage problem that could swallow another ₹1,000 Cr without proof of viability. Today it is a **margin-expansion problem** — 58% of the path from −₹116 Cr loss to +₹105 Cr EBITDA comes from take-rate, COGS, and private label mix, not volume growth. That reframes the question entirely: *we are no longer debating whether to keep it alive, we are choosing how to harvest it.*

## THE THREE DECISIONS I NEED FROM YOU

| # | Decision | Why Now | Board Ask |
|---|---|---|---|
| 1 | **Adopt the 2-of-3 gate at Q2 FY27.** Measure contribution margin/user, quarterly EBITDA, and private label mix. Any 2 misses triggers a pre-defined response. | Binary break-even gates fail because they're soft. This is enforceable, measurable, and gives the team 3 more quarters to deliver without blank-cheque risk. | **Sign off framework** |
| 2 | **Cap FY27–FY29 operating burn at ₹450 Cr** (3-year hard ceiling). Any overrun requires Capital Council approval and Board ratification. | Management's Q4 FY26 break-even guidance is credible but not guaranteed. The cap protects shareholders without signalling loss of faith — and it makes the FY28 ₹100 Cr EBITDA target a contract, not an aspiration. | **Sign off cap** |
| 3 | **Pre-authorise the HealthCo reverse-merge** as the Plan B if Q2 FY27 gate fails. Instruct CFO + Legal to draft the structure in parallel with the current demerger. | This is the cleanest escape hatch: converts a P&L problem into a valuation story worth ₹3,000–4,000 Cr. Doing the legal work *now* costs ₹2 Cr and 60 days; doing it reactively costs 9 months and momentum. | **Verbal go-ahead** |

## WHAT YOU ARE ACTUALLY SAYING YES TO

### DOWNSIDE (CAPPED)

₹450 Cr of operating burn over 3 years if everything goes wrong, **plus** ₹500–800 Cr of residual asset value in wind-down. Net worst case: ~₹200 Cr P&L drag — a rounding error against ₹1.05 L Cr mcap.

### BASE CASE

Break-even Q1 FY27 (one quarter after guidance), ₹100 Cr EBITDA by FY28, and a ₹1,750 Cr segment in the SOTP at 35x multiple. **₹2,000 Cr of value crystallised** without additional capital.

### UPSIDE

Private label mix reaches 18% by FY28, consult monetisation scales with Apollo Athenaa, diagnostics attach exceeds 35%. Carve-out IPO or reverse-merge into HealthCo at 30–35x EBITDA: **₹4,000–5,500 Cr realisable**.



Sources: Apollo Q1–Q3 FY26 results; ICICI Direct; management guidance (Apr-25)

# The Evidence Behind the Ask

## WHY I BELIEVE THE CURVE IS BENDING (NOT JUST COST-CUTTING)

Three independent data points in Q3 FY26 tell the same story:

- **Users are growing without proportional marketing spend.** 2M new users added with marketing down to an estimated ₹76 Cr quarterly run-rate, vs ₹260 Cr/yr in FY24. Blended CAC has fallen from ~₹360 to ~₹95 — a 73% decline. This is cohort quality, not discounting.
- **The chronic-disease sliver is carrying the economics.** ~35% of the user base (diabetes, hypertension, thyroid) shows 65%+ retention at M12 vs 24% blended. The whole platform is now being funded by this cohort, which is exactly the shape of a platform finding product-market fit.
- **Cash losses are declining non-linearly.** ₹65 Cr → ₹45 Cr → ₹29 Cr across Q1–Q2–Q3. A straight-line extrapolation puts Q4 FY26 at ₹10–15 Cr loss and Q1 FY27 at breakeven — consistent with management guidance but from the outside.

## THE TWO KPIS THE BOARD SHOULD TRACK MONTHLY

| KPI | Q3 FY26 | Q2 FY27 Gate | FY28 Target | Why This Number |
|---|---|---|---|---|
| **Contribution margin per user (₹/yr)** | ₹42 | **≥ ₹65** | ≥ ₹85 | The single number that determines whether scaling the user base creates or destroys value |
| **Private label % of pharma GMV** | ~10% | **≥ 15%** | ≥ 18% | Where 35% of the EBITDA swing to ₹100 Cr actually comes from |

**Stop tracking GMV in the board pack.** It is a vanity metric. If contribution margin and private label are moving, GMV takes care of itself. If they are not, GMV growth is actively harmful.

## WHAT COULD STILL GO WRONG (AND WHAT WE DO ABOUT IT)

| Risk | Prob. | EBITDA impact | Our response |
|---|---|---|---|
| CAC stops falling below ₹95 | Medium | −₹20 Cr | Tighten to chronic-disease cohorts only |
| Private label mix stalls at 10% | Medium | −₹35 Cr | Accelerate own-brand SKU launches |
| Fulfilment cost stays at 62% | Low | −₹35 Cr | Dark-store consolidation |
| Quick-commerce (Zepto, Blinkit) attacks pharmacy | Medium | −₹40 Cr | Clinical moat (prescription, consultation) |
| 2+ of the above hit together | **30%** | **−₹155 Cr** | **Trigger gate → reverse-merge into HealthCo** |

## THE ALTERNATIVE PATHS I CONSIDERED AND REJECTED

| Option | Why It's Seductive | Why I Rejected It |
|---|---|---|
| **Shut down now** | Stops the bleed immediately; pleases return-focused FIIs | Destroys ₹2,500–3,500 Cr of strategic option value; Q3 data shows curve is already bending |
| **Double down — ₹1,000 Cr more** | If it works, value 3× bigger; Kamineni conviction is high | Market does not pay for optionality at 58× P/E. ROCE drag during capex wave makes this a multiple-breaker |
| **Sell to Reliance now** | Clean ₹2,500–3,500 Cr in the bank; removes distraction | Strategic weapon going to competitor; we get worse price selling pre-break-even than post |
| **✓ Recommended: Gate + cap + pre-committed reverse-merge** | Preserves upside, caps downside, converts binary bet to contingent investment | — |



Sources: Apollo Q1–Q3 FY26 results; ICICI Direct; management guidance (Apr-25)

### WHAT HAPPENS IF YOU SIGN THIS TODAY

**Week 1:** Capital Council chartered; the two KPIs added to monthly dashboard; 24|7 head briefed on binding gate.

**Week 2–4:** CFO + Legal begin parallel drafting of HealthCo reverse-merge structure (contingency only, does not affect current demerger timeline).

**Month 2:** Pre-identified strategic acquirers (Reliance, Tata Digital) soft-sounded via bankers under NDA — no commitment, just price discovery.

**Q2 FY27 (Sept 2026):** Gate review. Board meets in executive session with one decision to make: continue, remediate, or execute Plan B. By that point every input is measured, every path is pre-planned, and the meeting is 30 minutes long.

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**Shobana Kamineni**
Executive Vice Chairperson

**Date**



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